

## **Dental Bridging Antibiotic**

Patient Group Direction, version 003, issued 8 September 2026. Amoxicillin for most patients, metronidazole where penicillin-allergic. Adults 18 and over.

### **What this service is, and is not**

- **This is a BRIDGE to dental care, not treatment. The definitive treatment for a dental infection is dental: drainage, extraction or root canal. An antibiotic buys a few days.**
- **Do not supply unless the patient is willing and able to arrange an urgent dental appointment within 24 to 48 hours.**
- **ONE COURSE PER EPISODE. A patient who has already had an antibiotic for this tooth is referred, not re-supplied.**
- **ANY facial swelling refers. There is no such thing as a minor facial swelling in dental infection.**
- **This PGD does not provide analgesia. Where the patient needs pain relief, offer pharmacy-sale analgesia under protocol and record it.**

### **The penicillin-allergic arm is new in this version**

- **Version 002 authorised amoxicillin and nothing else, and excluded penicillin-allergic patients altogether. That left roughly one patient in ten with a dental infection, pain and no route.**
- **METRONIDAZOLE 200mg THREE TIMES DAILY is now authorised for those patients. Read the dose again: 200mg, not 400mg.**
- **The dose matters and is worth stating plainly. The SPC for metronidazole gives acute dental infection as 200mg three times daily for 3 to 7 days. SDCEP guidance suggests 400mg 8-hourly, which is a higher dose than the licence carries for this indication. This PGD uses the licensed 200mg so that supply is within the marketing authorisation and needs no off-label consent. A pharmacist who has seen 400mg elsewhere is not wrong about the guidance; this document is simply staying inside the licence.**
- **Take the allergy history properly. Which arm applies turns on it, and "penicillin gave me an upset stomach" is an intolerance, not an allergy.**

## Amoxicillin, for patients who are not penicillin-allergic

**Patient Group Direction for the supply of amoxicillin 500mg capsules as bridging antibiotic care for a clearly localised acute dental infection in an adult awaiting urgent dental assessment.**

### Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC.
- Must have completed training relevant to this service, documented and overseen by the Get Real Health team.
- Must be familiar with the current SPC for the product supplied and with the BNF.
- Must be competent in recognising the dental red flags in Appendix 1, and in particular in identifying facial swelling and airway compromise.
- **MUST BE ABLE TO TAKE A RELIABLE PENICILLIN ALLERGY HISTORY** and to distinguish a true allergy from an intolerance. Which arm of this PGD applies turns entirely on that answer.
- Must understand that metronidazole and alcohol together cause a disulfiram-like reaction, and be able to explain the 48 hour rule so that the patient actually follows it.
- Must previously have used PGDs to supply medication.
- Must work in compliance with the SOPs of their own employer and practise only within the bounds of their own competence.
- All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

### The PGD

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| <b>Indication</b>         | Bridging antibiotic care for a clearly localised acute dental infection in an adult aged 18 years and over who is awaiting urgent dental assessment. This is not definitive treatment.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            |
| <b>Inclusion criteria</b> | <ul style="list-style-type: none"><li>• Adults aged 18 years and over.</li><li>• A clearly localised dental infection: pain, tenderness, localised gum inflammation or purulent discharge confined to the tooth and adjacent gum.</li><li>• <b>NO facial swelling of any kind.</b></li><li>• <b>NO systemic features: temperature below 38C, and no malaise, rigors or feeling generally unwell.</b></li><li>• Willing and able to arrange an urgent dental appointment within 24 to 48 hours.</li><li>• <b>NOT penicillin-allergic. Where the patient is, use the metronidazole arm.</b></li><li>• Capable of giving valid informed consent, and consent given.</li><li>• No exclusion criterion present, and no red flag from Appendix 1.</li></ul>                                                                                                                                                                                                                                                                                                                                             |
| <b>Exclusion criteria</b> | <p><b>Refer, do not supply, where any of the following is present.</b></p> <ul style="list-style-type: none"><li>• <b>ANY facial swelling, however localised or minor it appears.</b></li><li>• Any red flag listed in Appendix 1.</li><li>• Temperature of 38C or above, or malaise, rigors or systemic illness.</li><li>• Under 18 years of age. Children with dental infection are referred; this PGD provides no paediatric dose, no suitable formulation and no paediatric red-flag route.</li><li>• <b>Known penicillin or beta-lactam allergy, or any history of cephalosporin allergy. Consider the metronidazole arm.</b></li><li>• Known significant renal impairment. Refer. A five-day bridging course is not the place for a dose adjustment.</li><li>• Infectious mononucleosis or acute lymphoblastic leukaemia, because of the risk of rash.</li><li>• Already taking an antibiotic for this or any other indication.</li><li>• A course of antibiotic already supplied for this episode. One supply per episode only.</li><li>• Unable to give valid informed consent.</li></ul> |
| <b>Cautions</b>           | <b>Advise the patient to complete the course even if the pain settles, and</b>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                    |

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|                                                       | <p><b>that finishing it does not remove the need for the dental appointment.</b></p> <p>Diarrhoea is common. Advise the patient to seek advice for severe or bloody diarrhoea, which may indicate <i>Clostridioides difficile</i>.</p> <p>Where analgesia is appropriate and ibuprofen is unsuitable, supply paracetamol per the OTC pathway and record the reason.</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                        |
| <b>Actions if the patient is excluded or declines</b> | <p>Explain why supply is not appropriate and what to do instead. Where facial swelling is present arrange same-day medical or dental assessment rather than a routine appointment. Where the patient is penicillin-allergic, consider the metronidazole arm before referring. Document the advice given and the decision reached.</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          |
| <b>Red flags</b>                                      | <p><b>Refer, do not supply, where ANY of these is present. The first three need same-day urgent assessment, not a dental appointment.</b></p> <ul style="list-style-type: none"> <li>• <b>ANY facial swelling, however localised or minor it appears. Including swelling of the cheek, jaw, floor of mouth, or around the eye.</b></li> <li>• <b>Difficulty breathing, difficulty swallowing, drooling, or a changed voice. Airway compromise. Call 999.</b></li> <li>• <b>Swelling under the tongue or in the floor of the mouth, or a raised tongue. Suspect Ludwig angina. Same-day emergency.</b></li> <li>• Trismus, meaning difficulty opening the mouth.</li> <li>• Temperature of 38C or above, rigors, malaise, or the patient appearing systemically unwell.</li> <li>• Eye involvement: swelling around the eye, double vision, or reduced vision.</li> <li>• Spreading redness of the face or neck.</li> <li>• Confusion or drowsiness.</li> </ul> |

## The medicine

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| <b>Name, form and strength</b> | Amoxicillin 500mg capsules.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                 |
| <b>Legal category</b>          | POM.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                        |
| <b>Dose and frequency</b>      | 500mg three times daily for 5 days.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                         |
| <b>Quantity supplied</b>       | 15 capsules. A 5 day course. No repeat supply under this PGD.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                               |
| <b>Route and method</b>        | Oral.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       |
| <b>Storage</b>                 | Store below 25 degrees Celsius in the original packaging.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                   |
| <b>Adverse effects</b>         | Common: nausea, diarrhoea, rash. Uncommon: vomiting. Rare but serious: anaphylaxis, severe cutaneous reactions, antibiotic-associated colitis, cholestatic jaundice.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                        |
| <b>Yellow Card reporting</b>   | Report suspected adverse reactions via <a href="https://yellowcard.mhra.gov.uk">https://yellowcard.mhra.gov.uk</a> and inform the GP.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       |
| <b>Records to be kept</b>      | <ul style="list-style-type: none"> <li>• That valid informed consent was given.</li> <li>• Patient name, address, date of birth, and the GP with whom they are registered.</li> <li>• <b>THE PENICILLIN ALLERGY HISTORY, in the patient's own terms, and which arm was therefore used.</b></li> <li>• <b>That every red flag in Appendix 1 was asked about or looked for, and was absent.</b></li> <li>• <b>The dental appointment arranged, and when.</b></li> <li>• Name, form, strength, dose, quantity and date of supply.</li> <li>• <b>Where metronidazole was supplied, that the alcohol rule was explained, including the 48 hours after the course.</b></li> </ul> |

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|                 | <ul style="list-style-type: none"> <li>• Name and registration number of the healthcare professional supplying.</li> <li>• Advice given, including advice given if the patient is excluded or declines.</li> <li>• Details of any adverse drug reactions and the actions taken.</li> <li>• That the medicine was supplied under this PGD.</li> </ul> <p>Records signed, dated, legible and contemporaneous. Electronic records are acceptable. Keep for 8 years.</p> |
| <b>Disposal</b> | Advise the patient to return unused capsules to a pharmacy.                                                                                                                                                                                                                                                                                                                                                                                                          |

### Information for the patient

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| <b>Written information</b> | Supply the amoxicillin patient information leaflet. Do not supply the ibuprofen leaflet; ibuprofen is not supplied under this PGD.                                                                                                                                                                                                              |
| <b>Counselling</b>         | <ul style="list-style-type: none"> <li>• <b>This antibiotic is a bridge, not a cure. You must see a dentist within 24 to 48 hours.</b></li> <li>• One capsule three times a day for 5 days. Finish the course.</li> <li>• <b>Come back or seek urgent help if your face starts to swell, you cannot swallow, or you feel unwell.</b></li> </ul> |
| <b>Follow-up</b>           | Seek urgent help the same day for facial swelling, difficulty swallowing or breathing, or feeling generally unwell. Otherwise keep the dental appointment.                                                                                                                                                                                      |

## Metronidazole, for penicillin-allergic patients

**Patient Group Direction for the supply of metronidazole 200mg tablets as bridging antibiotic care for a clearly localised acute dental infection in a penicillin-allergic adult awaiting urgent dental assessment.**

*New in version 003. Version 002 excluded penicillin-allergic patients entirely, while the consultation tool was recommending metronidazole 400mg three times daily with nothing authorising it.*

### Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC.
- Must have completed training relevant to this service, documented and overseen by the Get Real Health team.
- Must be familiar with the current SPC for the product supplied and with the BNF.
- Must be competent in recognising the dental red flags in Appendix 1, and in particular in identifying facial swelling and airway compromise.
- **MUST BE ABLE TO TAKE A RELIABLE PENICILLIN ALLERGY HISTORY** and to distinguish a true allergy from an intolerance. Which arm of this PGD applies turns entirely on that answer.
- Must understand that metronidazole and alcohol together cause a disulfiram-like reaction, and be able to explain the 48 hour rule so that the patient actually follows it.
- Must previously have used PGDs to supply medication.
- Must work in compliance with the SOPs of their own employer and practise only within the bounds of their own competence.
- All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

### The PGD

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| Indication         | Bridging antibiotic care for a clearly localised acute dental infection in a penicillin-allergic adult aged 18 years and over who is awaiting urgent dental assessment. Acute dental infection is a licensed indication for metronidazole.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                        |
| Inclusion criteria | <ul style="list-style-type: none"><li>• Adults aged 18 years and over.</li><li>• A clearly localised dental infection, as for the amoxicillin arm.</li><li>• <b>NO facial swelling of any kind, and NO systemic features.</b></li><li>• Willing and able to arrange an urgent dental appointment within 24 to 48 hours.</li><li>• <b>A genuine penicillin or beta-lactam allergy, established by history and recorded.</b></li><li>• <b>ABLE AND WILLING TO AVOID ALCOHOL COMPLETELY during the course and for 48 hours afterwards. Ask directly. Where the patient says they cannot or will not, do not supply.</b></li><li>• Capable of giving valid informed consent, and consent given.</li><li>• No exclusion criterion present, and no red flag from Appendix 1.</li></ul>                                                                                                                                                                                                                                                                                                                                                  |
| Exclusion criteria | <p><b>Refer, do not supply, where any of the following is present.</b></p> <ul style="list-style-type: none"><li>• <b>ANY facial swelling, any red flag in Appendix 1, temperature 38C or above, or systemic illness. As for the amoxicillin arm.</b></li><li>• Under 18 years of age.</li><li>• Known hypersensitivity to metronidazole or to any nitroimidazole.</li><li>• <b>TAKING WARFARIN OR ANY COUMARIN ANTICOAGULANT. Metronidazole potentiates them and the INR rises. Refer.</b></li><li>• <b>TAKING LITHIUM. Lithium retention and renal damage have been reported. Refer.</b></li><li>• <b>TAKING DISULFIRAM, or having taken it in the last 2 weeks. Psychotic reactions have been reported with the combination.</b></li><li>• Taking busulfan, 5-fluorouracil or ciclosporin.</li><li>• Taking phenytoin or phenobarbital. These greatly increase metronidazole clearance and the course may be ineffective.</li><li>• Taking any medicine that prolongs the QT interval.</li><li>• <b>COCKAYNE SYNDROME. Severe and sometimes fatal hepatotoxicity with very rapid onset has been reported. Do not</b></li></ul> |

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|                                                       | <p>supply under this PGD in any circumstances.</p> <ul style="list-style-type: none"> <li>• <b>Unwilling or unable to avoid alcohol for the course and 48 hours afterwards.</b></li> <li>• <b>Pregnant or breastfeeding. The SPC advises metronidazole should not be given in pregnancy or lactation unless considered essential, which is a prescriber judgement and not a PGD one. Refer.</b></li> <li>• Active or chronic severe peripheral or central nervous system disease, because of the risk of neurological aggravation.</li> <li>• Known hepatic impairment.</li> <li>• Already taking an antibiotic, or a course already supplied for this episode.</li> <li>• Unable to give valid informed consent.</li> </ul>                                                                                                                                             |
| <b>Alcohol: how to ask, and what to say</b>           | <p><b>This is the single most important counselling point in this arm, and the commonest reason it goes wrong.</b></p> <p><b>Ask: "You must not drink any alcohol at all while taking these, and for two full days after the last tablet. Together they can make you very sick, with flushing, vomiting and a racing heart. Is that going to be a problem for you over the next week?"</b></p> <ul style="list-style-type: none"> <li>• A patient who hesitates, or who has an event coming up, should be told plainly and offered a referral instead.</li> <li>• Remember alcohol in things other than drinks: some mouthwashes, cough syrups and tonics.</li> <li>• <b>Record that the rule was explained and that the patient accepted it.</b></li> </ul>                                                                                                             |
| <b>Cautions</b>                                       | <p><b>Metronidazole darkens the urine. This is harmless and expected, but tell the patient or they will worry.</b></p> <p><b>May cause drowsiness, dizziness, vertigo or confusion. Advise the patient not to drive or operate machinery if affected.</b></p> <p><b>Advise the patient to stop and seek advice for any rash, blistering or peeling of the skin, or ulceration of the mouth. Severe bullous reactions including Stevens-Johnson syndrome, toxic epidermal necrolysis and acute generalised exanthematous pustulosis have been reported.</b></p> <p>Advise the patient to stop and seek advice for numbness or tingling in the hands or feet.</p> <p>Take the tablets with or after food, swallowed with water and not chewed.</p> <p>Complete the course even if the pain settles, and keep the dental appointment.</p>                                   |
| <b>Actions if the patient is excluded or declines</b> | <p>As for the amoxicillin arm. Where the exclusion is an interacting medicine, alcohol, or pregnancy, say plainly that the issue is the antibiotic and not the dental problem, and make sure the dental referral still happens. A penicillin-allergic patient who cannot have metronidazole still needs their tooth treated and may need a different antibiotic from a prescriber.</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                   |
| <b>Red flags</b>                                      | <p><b>Refer, do not supply, where ANY of these is present. The first three need same-day urgent assessment, not a dental appointment.</b></p> <ul style="list-style-type: none"> <li>• <b>ANY facial swelling, however localised or minor it appears. Including swelling of the cheek, jaw, floor of mouth, or around the eye.</b></li> <li>• <b>Difficulty breathing, difficulty swallowing, drooling, or a changed voice. Airway compromise. Call 999.</b></li> <li>• <b>Swelling under the tongue or in the floor of the mouth, or a raised tongue. Suspect Ludwig angina. Same-day emergency.</b></li> <li>• Trismus, meaning difficulty opening the mouth.</li> <li>• Temperature of 38C or above, rigors, malaise, or the patient appearing systemically unwell.</li> <li>• Eye involvement: swelling around the eye, double vision, or reduced vision.</li> </ul> |

- Spreading redness of the face or neck.
- Confusion or drowsiness.

## The medicine

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| <b>Name, form and strength</b> | Metronidazole 200mg tablets.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                  |
| <b>Legal category</b>          | POM.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          |
| <b>Dose and frequency</b>      | 200mg THREE TIMES DAILY for 5 days. This is the licensed dose for acute dental infection, which the SPC gives as 200mg three times daily for 3 to 7 days.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                     |
| <b>Quantity supplied</b>       | 15 tablets. A 5 day course. No repeat supply under this PGD.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                  |
| <b>Route and method</b>        | Oral. Swallow with water, with or after food. Do not chew.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                    |
| <b>A note on 400mg</b>         | SDCEP guidance for dental abscess suggests metronidazole 400mg 8-hourly. That is above the dose the SPC carries for acute dental infection and would be an off-label supply. This PGD deliberately authorises the licensed 200mg dose only. Do not substitute 400mg tablets or double the dose.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                               |
| <b>Storage</b>                 | Do not store above 25 degrees Celsius. Store in the original container or package.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            |
| <b>Drug interactions</b>       | Handled as exclusions rather than cautions: alcohol, disulfiram, warfarin and other coumarins, lithium, phenytoin, phenobarbital, 5-fluorouracil, ciclosporin, busulfan, and any QT-prolonging medicine. Check the current SPC and BNF against the patient's full medicine list, including anything bought over the counter.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                  |
| <b>Adverse effects</b>         | Not known frequency: taste disturbance, furred tongue, oral mucositis, nausea, vomiting, epigastric pain, diarrhoea. Very rare: darkening of urine, encephalopathy, subacute cerebellar syndrome, drowsiness, dizziness, convulsions, headache, psychotic disorders including confusion and hallucinations, vision disorders, agranulocytosis, neutropenia, thrombocytopenia, pancytopenia, hepatitis and jaundice, pancreatitis, skin rashes and pustular eruptions. Rare: anaphylaxis. Not known: peripheral sensory neuropathy with prolonged therapy, aseptic meningitis, hearing loss, tinnitus, QT prolongation, erythema multiforme, Stevens-Johnson syndrome and toxic epidermal necrolysis. Severe and sometimes fatal hepatotoxicity has been reported in Cockayne syndrome.                                                                                                                                        |
| <b>Yellow Card reporting</b>   | Report suspected adverse reactions via <a href="https://yellowcard.mhra.gov.uk">https://yellowcard.mhra.gov.uk</a> and inform the GP.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                         |
| <b>Records to be kept</b>      | <ul style="list-style-type: none"> <li>• That valid informed consent was given.</li> <li>• Patient name, address, date of birth, and the GP with whom they are registered.</li> <li>• <b>THE PENICILLIN ALLERGY HISTORY, in the patient's own terms, and which arm was therefore used.</b></li> <li>• <b>That every red flag in Appendix 1 was asked about or looked for, and was absent.</b></li> <li>• <b>The dental appointment arranged, and when.</b></li> <li>• Name, form, strength, dose, quantity and date of supply.</li> <li>• <b>Where metronidazole was supplied, that the alcohol rule was explained, including the 48 hours after the course.</b></li> <li>• Name and registration number of the healthcare professional supplying.</li> <li>• Advice given, including advice given if the patient is excluded or declines.</li> <li>• Details of any adverse drug reactions and the actions taken.</li> </ul> |

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|                 | <ul style="list-style-type: none"> <li>• That the medicine was supplied under this PGD.</li> </ul> Records signed, dated, legible and contemporaneous. Electronic records are acceptable. Keep for 8 years. |
| <b>Disposal</b> | Advise the patient to return unused tablets to a pharmacy.                                                                                                                                                  |

### Information for the patient

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| <b>Written information</b>           | Supply the metronidazole patient information leaflet.                                                                                                                                                                                                                                                                                                                                                                                                                                                               |
| <b>Counselling</b>                   | <ul style="list-style-type: none"> <li>• <b>NO ALCOHOL AT ALL while taking these, and for two full days after the last tablet. Together they can make you very unwell.</b></li> <li>• One tablet three times a day for 5 days, with or after food. Finish the course.</li> <li>• Your urine may go darker. That is expected and harmless.</li> <li>• <b>This is a bridge, not a cure. You must see a dentist within 24 to 48 hours.</b></li> <li>• Do not drive if the tablets make you drowsy or dizzy.</li> </ul> |
| <b>Follow-up and urgent symptoms</b> | <p><b>Stop the tablets and seek help the same day for:</b></p> <ul style="list-style-type: none"> <li>• <b>Any rash, blistering or peeling skin, or mouth ulcers.</b></li> <li>• Numbness or tingling in the hands or feet.</li> <li>• Confusion, unsteadiness or a fit.</li> </ul> <p><b>Seek urgent help at any time for facial swelling, difficulty swallowing or breathing, or feeling generally unwell.</b></p>                                                                                                |



## Appendix 1: Dental red flags. Ask and look before supplying anything

Any one of these excludes both arms. The first three are emergencies, not dental appointments.

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| Facial swelling of any kind                                 | <ul style="list-style-type: none"><li>Cheek, jaw, floor of mouth, or around the eye.</li><li>REFER SAME DAY. There is no minor facial swelling in dental infection.</li></ul> |
| Difficulty breathing or swallowing, drooling, changed voice | <ul style="list-style-type: none"><li>Airway compromise. CALL 999.</li></ul>                                                                                                  |
| Swelling under the tongue, or a raised tongue               | <ul style="list-style-type: none"><li>Suspect Ludwig angina. Same-day emergency.</li></ul>                                                                                    |
| Trismus                                                     | <ul style="list-style-type: none"><li>Difficulty opening the mouth. Refer same day.</li></ul>                                                                                 |
| Fever 38C or above, rigors, systemic illness                | <ul style="list-style-type: none"><li>Refer same day.</li></ul>                                                                                                               |
| Eye involvement                                             | <ul style="list-style-type: none"><li>Periorbital swelling, double vision, reduced vision. Refer same day.</li></ul>                                                          |
| Spreading facial or neck redness                            | <ul style="list-style-type: none"><li>Refer same day.</li></ul>                                                                                                               |
| Confusion or drowsiness                                     | <ul style="list-style-type: none"><li>Refer same day.</li></ul>                                                                                                               |

## Appendix 2: Key references

- Summary of Product Characteristics for amoxicillin 500mg capsules and for metronidazole 200mg tablets, current versions.
- Scottish Dental Clinical Effectiveness Programme, Drug Prescribing for Dentistry, current edition.
- British National Formulary, and BNF guidance on oral bacterial infections.
- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions, <https://www.nice.org.uk/guidance/mpg2>

## Authorisation

|                 |                               |
|-----------------|-------------------------------|
| Version         | v003                          |
| Supersedes      | Version 002, 7 September 2026 |
| Valid from date | 8 September 2026              |
| Expiry date     | 31 July 2027                  |

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| Doctor     | Name: Nitin Shori<br>Job title: Medical Director, Get Real Health<br>GMC: 6047293<br><b>Signed: N. Shori</b><br>Date: 8 September 2026 |
| Pharmacist | Name: Chris Pilkington<br>Job title: Head Pharmacist, Get Real Health<br>GPhC: 2046322<br><b>Signed: C. Pilkington</b>                 |

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|  | Date: 8 September 2026 |
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Both authorising signatories reviewed this version together on 8 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs.

**Adoption by the employing organisation**

To be completed by the adopting pharmacy. These signatories are the adopting organisation own signatories and must not be pre-filled by Get Real Health.

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| <b>Superintendent / clinical lead</b> | Name: .....<br>Job title and organisation: .....<br>Signature: .....<br>Date: ..... |
| <b>Professional group signatory</b>   | Name: .....<br>Job title and organisation: .....<br>Signature: .....<br>Date: ..... |

## Change history

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| <b>Version</b> | v003                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                               |
| <b>Date</b>    | 8 September 2026                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                   |
| <b>Changes</b> | <ul style="list-style-type: none"> <li>• A metronidazole arm is added for penicillin-allergic adults. Version 002 authorised amoxicillin only and excluded penicillin-allergic patients altogether, which left roughly one patient in ten with a dental infection and no route.</li> <li>• The dose is metronidazole 200mg three times daily for 5 days. This is the licensed dose for acute dental infection, which the SPC gives as 200mg three times daily for 3 to 7 days.</li> <li>• The document states explicitly that SDCEP suggests 400mg 8-hourly, that this is above the licensed dose for the indication, and that this PGD deliberately stays within the licence. The ePGD tool had been recommending 400mg three times daily with no document authorising it at all, which is both an unauthorised medicine and a dose above the licence.</li> <li>• Alcohol handled as an inclusion criterion rather than a counselling point: the patient must be able and willing to avoid it completely for the course and 48 hours afterwards, must be asked directly, and the answer must be recorded.</li> <li>• Warfarin and other coumarins, lithium, disulfiram, busulfan, 5-fluorouracil, ciclosporin, phenytoin, phenobarbital and QT-prolonging medicines are exclusions in the metronidazole arm, not cautions.</li> <li>• Cockayne syndrome is an absolute exclusion, following reports of severe and sometimes fatal hepatotoxicity with very rapid onset.</li> <li>• Pregnancy and breastfeeding excluded from the metronidazole arm and referred, the SPC advising it should not be given unless considered essential, which is a prescriber judgement.</li> <li>• Records must now carry the penicillin allergy history in the patient's own terms, which arm was used and why, and confirmation that the alcohol rule was explained.</li> <li>• Red flags collected into Appendix 1 and applied to both arms, with the three that are emergencies rather than dental appointments identified as such.</li> </ul> |

## Previous versions

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| <b>001</b>                   | Development and issue of new PGD. Amoxicillin and ibuprofen arms.                                                                                                                                                                           |
| <b>002, 7 September 2026</b> | Full clinical review following Jack Ellison's Heron Cross letter. The ibuprofen arm was removed, ibuprofen being a P medicine for which a PGD is not required. Any facial swelling became a referral. Amoxicillin only, adults 18 and over. |